

BRAIN HEALTH AS A LABOUR-MARKET INPUT: AN EMERGENCE SIGNAL CHECK

Google Trends and OpenAlex publication data, single collection round, 21–22 July 2026

BRAIN HEALTH AS AN ECONOMIC INPUT — TECHNICAL BRIEFING

PREMISE

Eli Ifaturoti · eli.ifaturoti@oecd.org

POLICY QUESTION

Does discourse from public and unconventional datasets amplify academic evidence of female brain health being economically material?

OECD has already made the case that women's brain health is economically material: its New Approaches for Economic Challenges unit has run Brain Capital, a programme treating brain health as a core economic asset, since 2021, including work on sex and gender differences in neurological outcomes¹. What is not yet tested is whether public, unconventional data, search interest, publishing activity, amplifies that case: gives it earlier, cheaper, independently observable evidence than the institutional argument alone provides.

The labour-market cost that argument points to is already measured: menopausal symptoms are linked to higher sick leave and lower work productivity², adult ADHD to comparably large productivity loss³, and Long Covid to higher odds of leaving employment the longer symptoms persist⁴. If unconventional public data tracks the same conditions, it gives policymakers a faster, cheaper signal on top of the institutional case, not a competing one.

This proposal tests a narrower, answerable version of that question: for eight topics spanning the female lifespan, perimenopause and memory, adult ADHD in women, long COVID cognitive effects and others, is public search interest and academic publishing growing fast enough to count as that kind of evidence. This page reports that first check, using two free data sources, Google Trends and OpenAlex. It does not yet run the medical-recognition check itself, against MeSH, the step that would show whether a topic already has a formal classification.

Working hypothesis: these eight topics are gaining public and research attention faster than they are gaining formal medical recognition. The eight were chosen because each names a phase of the female lifespan or a related cognitive condition already raised in OECD's Brain Capital seminars, not because they scored highest in a broader keyword sweep.

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TOPICS WITH POSITIVE EMERGENCE

4

TOPICS SCORING ABOVE 8,000

OBJECTIVE, THIS ROUND

Confirm, from this single 21–22 July 2026 data pull, whether at least half of the eight topics show positive growth on both signals, enough to justify running the MeSH comparison next round.

LIMITATIONS

- No comparison against existing medical terminology yet.
- No combined priority score, only the emergence half.
- No trend line, one collection round only.

(1) Eyre et al. (2021), *Neuron*. (2) O'Neill, Jones & Reid (2023), *Occupational Medicine*. (3) Joseph et al. (2019), *Journal of Attention Disorders*. (4) Ayoubkhani et al. (2024), *European Journal of Public Health*. Full citations on page 3.

WHAT THE TWO DATA SOURCES SHOW

Continued

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FINDINGS

ALL EIGHT TOPICS ARE GROWING; FOUR STAND OUT

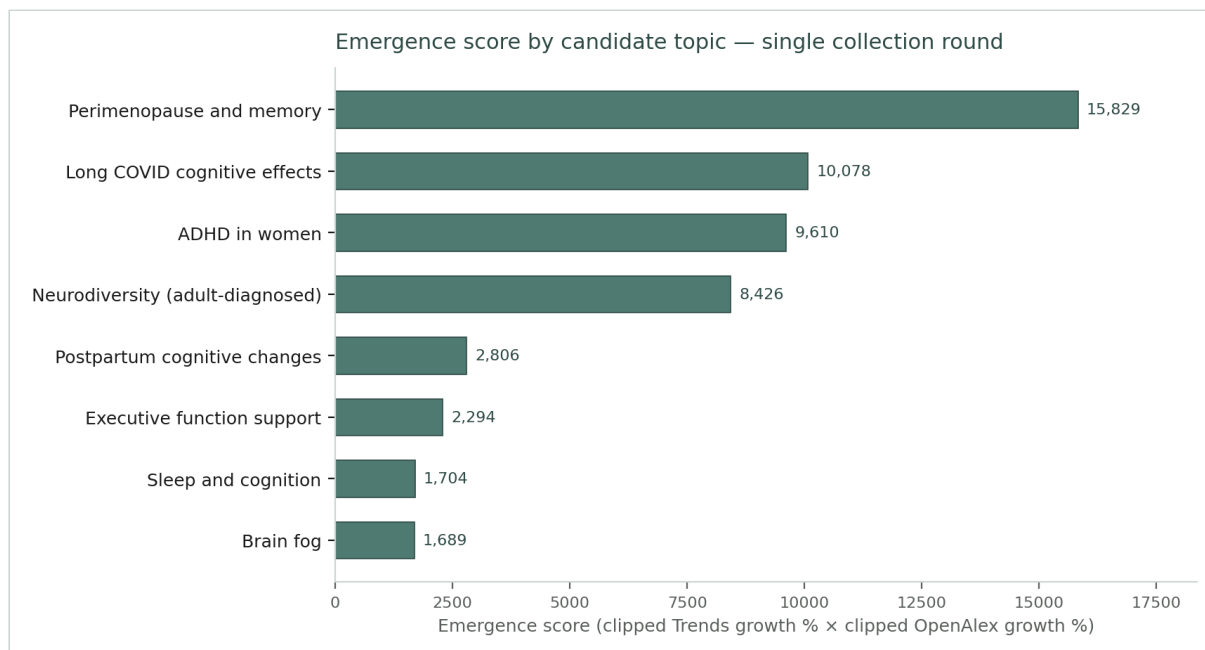
Perimenopause and memory, long COVID cognitive effects, ADHD in women and neurodiversity (adult-diagnosed) combine rising Google Trends search interest with rising OpenAlex publication counts at the same time, exactly the kind of convergence the emergence-score formula was designed to reward rather than either signal rising on its own. The remaining four topics are still growing on both signals, just less sharply.

Emergence score is calculated exactly as pre-registered: clipped Google Trends growth (recent 3-month mean vs. prior 12-month mean) multiplied by clipped OpenAlex publication growth (trailing 3-year works vs. prior 3-year works from a 6-year series), both terms floored at zero before multiplying¹. No window or formula choice was adjusted after seeing these numbers.

EXECUTIVE FUNCTION SUPPORT IS A KNOWN OUTLIER

Its OpenAlex counts (80,067 prior-period works, 97,559 recent) are an order of magnitude larger than every other topic's, because both of its keyword variants ("executive function", "executive function support") are generic clinical terms without a women's-health qualifier. That risk was written down as a limitation before the data was collected, not adjusted after seeing the numbers.

FIGURE 1 EMERGENCE SCORE BY CANDIDATE TOPIC



Source: own analysis, Google Trends (pytrends) / OpenAlex Works API, 21–22 July 2026.

(1) Both growth terms are clipped at zero before multiplying, a rule fixed before this data was collected, so that two co-declining signals cannot produce a false-positive emergence score.

EVIDENCE TABLE AND WHAT HAPPENS NEXT

Continued

EVIDENCE SUMMARY

EMERGENCE COMPONENTS BY TOPIC, RANKED BY EMERGENCE SCORE

Topic	Trends growth %	OpenAlex growth %	Emergence score
Perimenopause and memory	117.8	134.4	15,829
Long COVID cognitive effects	121.7	82.8	10,078
ADHD in women	100.1	96.0	9,610
Neurodiversity (adult-diagnosed)	67.6	124.6	8,426
Postpartum cognitive changes	133.3	21.1	2,806
Executive function support	105.0	21.8	2,294
Sleep and cognition	217.9	7.8	1,704
Brain fog	53.1	31.8	1,689

READING THE RANKING CORRECTLY

Sleep and cognition has the highest Trends growth (217.9%) but ranks near the bottom on emergence because its OpenAlex growth is flat (7.8%): the multiplicative formula rewards convergence across both signals over strength on either one alone. The same logic explains why postpartum cognitive changes, the highest Trends growth of all eight at 133.3%, still ranks fifth.

WHAT'S STILL MISSING

The medical-recognition check has not been run yet. This page compares public search interest against academic publishing only, not yet against MeSH, the US National Library of Medicine's controlled vocabulary, which would show whether a topic already has formal recognition. Adding that comparison would produce a single priority score per topic; without it, these numbers describe attention and publishing activity only.

DECISION ASK

Two options: add the medical-terminology comparison now for the four strongest-growth topics, or first repeat the count for all eight to confirm the ranking holds. The first is faster but rests on one data pull; the second costs a delay but checks against a one-off spike. Neither is clearly better, so the choice is left open here.

REFERENCES

Academic — Eyre et al. (2021). Building brain capital. *Neuron*, 109(9).

O'Neill, Jones & Reid (2023). Impact of menopausal symptoms on work and careers. *Occupational Medicine*, 73(6).

Joseph et al. (2019). Health-related quality of life and work productivity of adults with ADHD. *Journal of Attention Disorders*, 23(12).

Ayoubkhani et al. (2024). Employment outcomes of people with Long Covid symptoms. *European Journal of Public Health*, 34(3).

Institutional — OECD NAEC Unit, Neuroscience-inspired Policy Initiative, Brain Capital programme.

Data sources — Google Trends (pytrends); OpenAlex Works API, 21–22 July 2026.